MASSACHUSETTS

Blue Cross Blue Shield of Massachusetts is an Independent Licensee of the Blue Cross and Blue Shield Association

Pharmacy Medical Policy

Opioid and Opioid Combination Medication Management

Table of Contents

- Policy: Commercial
- Policy: Medicare
- Description
- Information Pertaining to All Policies
- Policy History
- References
- Forms

Policy Number: 102

BCBSA Reference Number: None

Related Policies

- Quality Care Dosing guidelines apply to the following medications and can be found in Medical Policy #621

Prior Authorization Information

☒ **Prior Authorization** ☐ **Step Therapy** ☒ **Quality Care Dosing**		**Pharmacy Operations:** Tel: 1-800-366-7778 Fax: 1-800-583-6289 Policy last updated **11/2/2026**
Pharmacy (Rx) or Medical (MED) benefit coverage	☒ **Rx** ☐ **MED**	**To request for coverage:** Physicians may call, fax, or mail the attached form (Formulary Exception/Prior Authorization form) to the address below. **Blue Cross Blue Shield of Massachusetts** **Pharmacy Operations Department** 25 Technology Place Hingham, MA 02043 **Individual Consideration:** Policy for requests that do not meet clinical criteria of this policy, see section labeled Individual Consideration
Policy applies to Commercial members with BCBSMA formulary: • Managed Care (HMO/POS) • PPO/EPO • Indemnity • MEDEX with Rx plans • Managed Blue for Seniors **Policy does NOT apply to**: • Medicare Advantage		

Please refer to the chart below for the formulary and step status of the medications affected by this policy.

Standard Formulary		
Short-Acting Opioids (including generic products where applicable) †		
Alfenta (alfentanil)	hydrocodone/APAP	Oxaydo (oxycodone)**
Apadaz (benzhydrocodone/APAP)**	hydrocodone/ibuprofen	oxycodone
benzhydrocodone/APAP	hydromorphone	oxycodone/APAP
codeine	Infumorph [morphine (systemic)]	oxymorphone
codeine/APAP	Levorphanol	Percocet (oxycodone/APAP)**
Demerol (meperidine)	Lorcet /HD /Plus (hydrocodone/APAP)	Primlev (oxycodone/APAP)
dihydrocodeine/APAP/caffeine	meperidine HCl	Prolate (oxycodone/APAP)
Dilaudid (hydromorphone) tabs**, - 5**, - HP, Injectable	morphine Sulfate	Roxicodone (oxycodone)
Duramorph PS [morphine (systemic)] **	Nalocet (oxycodone/APAP)	RoxyBond (oxycodone)
Dvorah (APAP/caff/dihydrocodeine)	Norco (hydrocodone/APAP)	Tapentadol solution
Endocet (oxycodone / APAP)	Nucynta (tapentadol)**	Trezix** (dihydrocodeine/APAP/caffeine)
fentanyl	oxycodone/aspirin	
Long-Acting Opioids (including generic products where applicable) †		
Belbuca (buprenorphine)	hydromorphone ER	Nucynta ER (tapentadol) **
buprenorphine Film	Hysingla ER (hydrocodone) **	oxycodone ER**
buprenorphine Patch	methadone	Oxycontin (oxycodone) **
Butrans (buprenorphine)	Methadose (methadone)	oxymorphone ER
fentanyl Patch	morphine sulfate	Xtampza ER (oxycodone
hydrocodone Bitartrate ER	MS Contin (morphine)	

PA *– Prior Authorization;* ***NFNC*** *– Non-formulary, Non-Covered;* ***QCD*** *(Quality Care Dosing – refer to Policy 621b)*

***Non-formulary medications are covered when a formulary exception request is submitted to BCBSMA Pharmacy Operations and criteria below are met.*

† New medications released to market in these classes will be added to the policy as part of the New Drug Approval process.

OPIOID AND OPIOID COMBINATION PRESCRIPTIONS ARE EXCLUDED THROUGH THE MAIL ORDER PHARMACY BENEFIT.

Description

Opioids are powerful tools for the relief of pain. They are however associated with the risks of addiction and diversion for non-medical purposes. The purpose of this policy is to use current medical literature to establish processes to promote the effective use of opioids in pain management and to prevent inadvertent addiction and deter diversion. It is our expectation that members using opioids will be compliant with this policy and that coverage for associated medical services is dependent on this compliance.

The use of opioids is associated with the risk of addiction. Informing patients of this risk is an important activity in the evidence-based use of opioids. Ideally this informed consent, with a risk assessment of addiction potential, is given and obtained prior to an initial prescription for any opioid. A treatment plan including a clear diagnosis, explicit goals, and exploration of other treatment options should exist.
If members are to receive opioids chronically, a written/signed agreement between prescriber and patient addressing issues of prescription management, diversion, and the use of other substances should exist.
Chronic prescribing of opioids should be done by a single designated prescriber group and medications

should be obtained by the member at one pharmacy or pharmacy chain.

Urine drug testing (UDT) is medically necessary and plays an integral role of effective medical management of opioids. It should be used appropriately to inform clinical decision making. See policy # 674 for additional details.

Buprenorphine HCl with naloxone is indicated for the treatment of opioid dependence. It should not be used in conjunction with Opioids as toxicity risk is increased. Exception may exist for things like short-term use for injuries and surgeries and should be incorporated into the Opioid Use Disorder (OUD) treatment plan if and when applicable.

In 2025, the FDA is requiring safety labeling changes to all opioid pain medications to better emphasize and explain the risks associated with their long-term use. Labeling changes include: clearer risk information, dosage warnings, clarified use limits, treatment guidance, safe discontinuation, overdose reversal agents, drug interactions, more risks with overdose, and digestive health. The FDA has also required an additional prospective, randomized, controlled clinical trial to directly examine the benefits and risks of long-term opioid use. The Agency will be closely monitoring the progress of this clinical trial to ensure its timely completion.

Please refer to Medical Policy #113 for coverage of oral/transmucosal Fentanyl products.

Quality Care Dosing guidelines may apply to the following medications and can be found in Medical Policy #621A.

Coverage Criteria: Short-Acting Medications

We may cover short-acting opioids listed in the chart above when the following criteria are met:

- The member has a diagnosis of cancer or Sickle Cell disease

OR

- The prescription is written by an oncologist, palliative care prescriber, pain management specialist, or oncology prescriber

OR

- The member is receiving opioids as part of palliative care.

For all other requests:

- **We may cover short-acting formulary opioids listed in the chart above for up to one initial 7 day supply without prior authorization.**
- **We may cover up to two additional 7 days supply of short-acting formulary opioids listed in the chart above within 60 days of the original prescription without prior authorization.**
- **After the initial three prescriptions or an excess of 21 days within 60 days, prior authorization IS required and we may cover additional treatment with short-acting formulary opioids within 60 days of the original prescription when the following criteria are met:**
  - The prescriber certifies there is an active treatment plan that includes but is not limited to a specific treatment objective and the use of other pharmacological and non-pharmacological agents for pain relief as appropriate.
  - The prescriber certifies there has been an informed consent document signed and an addiction risk assessment has been performed.
  - The prescriber certifies that a written/signed agreement between prescriber and patient addressing issues of prescription management, diversion, and the use of other substances exists.
  - The prescriber certifies that a prescribing group and a preferred pharmacy chain has been agreed upon by the prescriber and the member.

For non-formulary short acting opioids, requests must meet criteria above and the member must have had a previous treatment failure with or a contraindication to two covered formulary alternatives.

If approved the Prior Authorization will be granted for up to one (1) year for the specific opioid requested. This authorization may cover only the specific opioid requested. Non-formulary medications will not be covered unless a formulary exception has been approved.

Coverage Criteria: Long-Acting Medications

We may cover long-acting opioids listed in the chart above when the following criteria are met:

- The member has a diagnosis of cancer or Sickle Cell Disease**, OR**
- The prescription is written by an oncologist, palliative care prescriber, pain management specialist, or oncology prescriber, **OR**
- The member is receiving opioids as part of palliative care, **OR**

- The request meets **ALL** of the following criteria:
    - There has been previous treatment failure with a short-acting opioid with in the previous 60 days.
    - The prescriber certifies there is an active treatment plan that includes but is not limited to a specific treatment objective and the use of other pharmacological and non-pharmacological agents for pain relief as appropriate.
    - The prescriber certifies there has been an informed consent document signed and an addiction risk assessment performed.
    - The prescriber certifies that a written/signed agreement between prescriber and patient addressing issues of prescription management, diversion, and the use of other substances exists.
    - The prescriber certifies that a prescribing group and a preferred pharmacy chain has been agreed upon by the prescriber and the member.

For non-formulary long-acting opioids, requests must meet criteria above and has previous treatment failure with or a contraindication to two covered formulary alternatives.

If approved the Prior Authorization will be granted for up to one (1) year for the specific opioid requested. This authorization may cover only the specific opioid requested. Non-formulary medications will not be covered unless a formulary exception has been approved.

90 Morphine Milligram Equivalents (MME) Safety Edit

Effective July 1, 2021 BCBSMA will implement a real-time, point of service opioid safety edit to prevent unsafe dosing of drugs at the time of dispensing. The edit will provide real-time information to help ensure prescribers are aware/notified that their patient(s) may be receiving potentially high-risk levels of opioids. The Morphine Milligram Equivalent (MME) 90 Opioid Safety Edit is triggered at the pharmacy when a patient's total (cumulative) opioid dose across all opioid or opioid-containing prescriptions reaches or exceeds 90 MME per day.

The dispensing pharmacist will receive an alert to consult with the prescriber before proceeding with dispensing the medication. The pharmacist may issue an override code at the time of dispensing to process the claim and dispense the medication. It is important to note that even if the prescriber confirms intent, consultation with the prescriber does not supersede the dispensing pharmacist's professional judgement and decision to dispense or not dispense the prescription.

If the pharmacist is unable resolve the safety edit at the point of sale (e.g., prescriber could not be reached, prescriber was consulted but did not verify medical necessity, pharmacist exercises professional judgement and decides to not dispense the medication), the prescriber can request an exception for coverage thru BCBSMA Pharmacy Operations.

Authorization for coverage will be approved for up to 1 year if the patient meets ONE of the following criteria:

- The member has a diagnosis of cancer, **OR**
- The member is receiving opioids as part of palliative care**, OR**
- The prescription is written by an oncologist, palliative care prescriber, pain management specialist, or oncology prescriber, **OR**
- The prescriber states that based on the patient's clinical circumstances the amount of opioid prescribed is warranted in order to adequately manage the patient's pain.

The 90MME safety edit is consistent with CDC guidelines for prescribing opioids for chronic pain, which includes avoiding increasing opioids dosage ≥90MME/day or carefully justifying a decision to titrate dosage

to ≥90MME/day. More information on CDC guidelines for prescribing opioids for chronic pain may be found at:
https://www.cdc.gov/mmwr/volumes/65/rr/rr6501e1.htm?CDC_AA_refVal=https%3A%2F%2Fwww.cdc.gov%2Fmmwr%2Fvolumes%2F65%2Frr%2Frr6501e1er.htm

Appendix

Formulary Status	For non-covered medications, in addition to the prior authorization criteria, the member **must** also have had a previous treatment failure with, or contraindication to, **at least two** covered formulary alternatives when available. See section on individual consideration for more information if you require an exception to any of these criteria requirements for an atypical patient.
Member cost share consideration	A higher non-preferred cost share may be applied if an exception request is approved for coverage of a non-preferred or a non-formulary/non-covered drug.
Forms	To request prior authorization using the Massachusetts Standard Form for Medication Prior Authorization Requests (eForm), click the link below: **https://www.bluecrossma.org/medical-policies/sites/g/files/csphws2091/files/acquiadam-assets/023%20E%20Form%20medication%20prior%20auth%20instruction%20prn.pdf** **OR** **Print and fax, Massachusetts Standard Form for Medication Prior Authorization Requests** #434
Individual Consideration (for Atypical Patients)	Our medical policies are written for most people with a given condition. Each policy is based on peer reviewed clinical evidence. We also take into consideration the needs of atypical patient populations and diagnoses. If the coverage criteria outlined is unlikely to be clinically effective for the prescribed purpose, the health care provider may request an exception to cover the requested medication based on an individual's unique clinical circumstances. This is also referred to as "individual consideration" or an "exception request." Some reasons why you may need us to make an exception include: therapeutic contraindications; history of adverse effects; expected to be ineffective or likely to cause harm (physical, mental, or adverse reaction). To facilitate a thorough and prompt review of an exception request, we encourage the provider to include additional supporting clinical documentation with their request. This may include: • Clinical notes or supporting clinical statements; • The name and strength of formulary alternatives tried and failed (if alternatives were tried) and specifics regarding the treatment failure, if applicable; • Clinical literature from reputable peer reviewed journals; • References from nationally recognized and approved drug compendia such as American Hospital Formulary Service® Drug Information (AHFS-DI), Lexi-Drug, Clinical Pharmacology, Micromedex or Drugdex®; and • References from consensus documents and/or nationally sanctioned guidelines Providers may call, fax or mail relevant clinical information, including clinical references for individual patient consideration, to: Blue Cross Blue Shield of Massachusetts Pharmacy Operations Department 25 Technology Place Hingham, MA 02043 Phone: 1-800-366-7778 Fax: 1-800-583-6289

Policy History

Date	Action
11/2/2026	Added Tapentadol solution to the policy.
5/2026	Annual Review: Formatting, reference, and literature update. Removed obsolete brands. No changes to criteria.
11/2024	Clarified covered Diagnoses
7/2023	Reformatted Policy.
5/2023	Clarified use with SUD treatment.
7/2022	Updated tables with removal of discontinued drugs.
10/2021	Updated to add buprenorphine buccal film to short acting policy list.
7/2021	Updated to add 90 MME safety edit and make Oxycontin nonpreferred.
11/2020	Updated to clarify we will only approve authorizations for specific opioids and not for groups of opioids.
4/2020	Updated to include Hydrocodone Bitartrate ER Capsules & Prolate to the policy.
4/2019	Updated to include Benzhydrocodone/APAP & Apadaz® to the policy.
1/2019	Update to include Dvorah and to add a specialty provider type.
11/2018	Updated to include Nalocet™ & Roxybond™ into the short acting criteria.
10/2017	Updated to add AG Buprenorphine Patch & Butrans® to the formulary.
6/2017	Updated address for Pharmacy Operations and added Morphabond™ ER.
4/2017	Updated to add Arymo™ ER to the long acting part of the policy.
1/1/2017	Updated to make some format changes.
8/2016	Updated to align with Massachusetts Opioid law and shortened criteria for short-acting Opioids.
4/2016	Updated to add Belbuca to Long acting section of policy
8/2015	Updated Request form title
1/2015	Updated to remove UDT and added Zohydro ER and Verdocet
1/2014	Updated ExpressPAth language.
7/2012	New Policy, effective 7/1/12

References

1. Arnold RM, Han Pk, Seltzer D. Opioid contracts in chronic nonmalignant pain management: objectives and uncertainties. *Am J Med* 2006; 119:292-296.
2. Chou R. Fanciullo GJ, et al. Clinical guidelines for the use of chronic opioid therapy in chronic non cancer pain. *J Pain* 2009; 10: 113-1302.
3. Gourlay DL, Heit HA, Almahrezi A. Universal precautions in pain medicine: a rationale approach for the treatment of chronic pain. *Pain Med.* 2005; 6:107-112.
4. Interagency Guideline for Chronic Non-Cancer Pain: An Educational Aid to improve the care and safety of Opioid therapy: 2010 update; www.agencymeddirectors.wa.gov/Files/OpioidGdline.pdf
5. Model Policy for the Use of Controlled Substances for The Treatment of Pain; Federation of State Medical Boards of the United States, Inc.; http://www.fsmb.org/pdf/2004_grpol_Controlled_Substances.Pdf
6. Noble M. Long-term opioid management for chronic noncancer pain. *Cochrane Database of Systematic Reviews* [serial online]. November 3, 2009; (1) Available from: Cochrane Database of Systematic Reviews, Ipswich, MA. Accessed Augusts 10, 2010.
7. Opioid Treatment Guidelines: Clinical Guidelines for the Use of Opioid Therapy in Chronic Noncancer Pain; *The Journal of Pain*, Vol 10, No.2 (February), 2009 pp 113-130
8. Starrel JL, Becker WC, et.al. Systematic review: Treatment agreements and urine drug testing to reduce opioid misuse in patients with chronic pain. *Ann Intern Med* 2010; 152: 712-720.
9. Massachusetts Drug Formulary Commission (DFC): Non-Opioid Drug Products for Pain Management. http://www.mass.gov/eohhs/docs/dph/quality/drugcontrol/drug-formulary/non-opioid-drug-list-sept-2017.pdf
10. Dowell, D., Ragan, K. R., Jones, C. M., Baldwin, G. T., & Chou, R. (2022). CDC Clinical Practice Guideline for Prescribing Opioids for Pain — United States, 2022. *Morbidity and Mortality Weekly Report*, *71*(3), 1–95. https://doi.org/10.15585/mmwr.rr7103a1